

SIR CHARLES GAIRDNER OSBORNE PARK HEALTH CARE GROUP AMPHETAMINE SUBSTANCES WITHDRAWAL QUICK REFERENCE GUIDE		URN:		
		Family name:		
		Given name(s):		
		Gender:	DOB:	
Signs of intoxication	Withdrawal timeframe	Crash phase	Withdrawal phase	Extinction phase
Autonomic Increased blood pressure Increased body temperature Rapid or irregular heartbeat Excessive sweating Muscle rigidity Central Euphoria Agitation / confusion Thought disturbance Anxiety / panic Paranoia Psychotic features Acute behavioural disturbance	<i>Crash:</i> starts 12–24 hours after last dose and lasts 24–48 hours <i>Withdrawal:</i> starts 2–4 days after last use, peaks in severity over 7–10 days, then subsides over 2–4 weeks <i>Extinction:</i> lasts for weeks, and may persist for up to 12 months	Exhaustion Fatigue Generalised aches and pains <i>Sleep disturbance:</i> Increased sleep Insomnia Restlessness Flat mood or dysphoria Anxiety / agitation Low level craving Thought disturbance usually masked during crash	Fatigue Anhedonia Generalised aches and pains Headache <i>Sleep disturbance:</i> Vivid dreams Insomnia <i>Fluctuating mood and energy:</i> Irritability Restlessness Anxiety / agitation Poor concentration and attention Strong cravings <i>Thought disturbance:</i> Paranoid ideation Strange beliefs Misperceptions Hallucinations	<i>Episodic fluctuations in mood and energy levels:</i> Irritability Restlessness Anxiety / agitation Fatigue Lacking energy Anhedonia Episodic cravings Disturbed sleep Gradual resumption of normal mood
Assessment / management tools (once diagnosis established)				
Clinical assessment and examination Clinical tool examples: <i>currently there are no validated withdrawal scales for hospital settings</i>				
Acute withdrawal treatment models				
General principles	Symptom management	Pharmacotherapy	Inpatient withdrawal	
Most amphetamine withdrawal can be managed in the community with good support Regular review may be required to monitor progress	Hydration Multivitamins Anti-Emetics Simple analgesia Calm environment Psycho-social education	CONSIDER: Low dose diazepam for agitation and poor sleep for up to 7 days Antipsychotic (e.g. Olanzapine 2.5–10 mg daily) for paranoid or psychotic features (Grade C)	Acute withdrawal whilst admitted should be managed supportively	
Post-withdrawal treatment options				
Ongoing counselling and support / groups, GP / AMS Residential rehabilitation / therapeutic communities Consider an antidepressant if ongoing depressive symptoms (Grade D).				